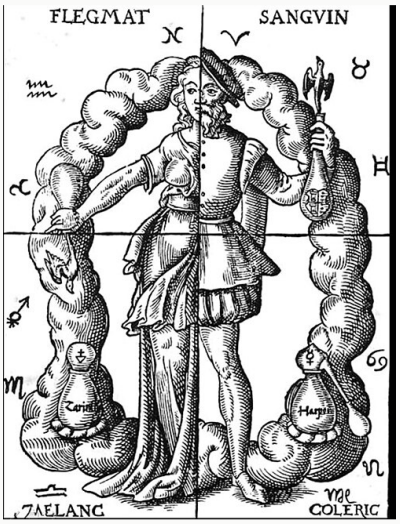
MASTER COURSE TEXTBOOK • THEMATIC STUDY

MEDIEVAL MEDICINE IN BRITAIN (c1250–c1500)

Supernatural Orthodoxy, The Galenic Monopoly & The Black Death

The History Department GCSE MASTERCLASS SERIES • C1250–C1500



Archival Photographic Plate • Contemporary Primary Record • Medieval Medicine in Britain

CORE SPECIFICATION ENQUIRIES

- **L1:** Supernatural & Religious Explanations
- **L2:** Rational Ideas: Galen & The Four Humours
- **L3:** Approaches to Treatment & Prevention
- **L4:** Medical Providers & Monastic Hospitals
- **L5:** Case Study: The Black Death (1348–49)

HISTORICAL MECHANISMS & CONCEPTS

- **Ecclesiastical Monopoly:** Scriptoria & Papal authority
- **Teleology:** Purposeful divine bodily design
- **Humoural Balance:** Opposites, phlebotomy & purging
- **Hospital Care:** *Care Not Cure* monastic regimes
- **Astrological Miasma:** 1345 Planetary conjunction

EDEXCEL PAPER 1 EXAMINATION BLUEPRINT

- **Q3:** Similarity / Difference across Eras [4m]
- **Q4:** Causal Analysis / Explain Why [12m]
- **Q5/Q6:** 16-Mark Synoptic Essay [+4 SPaG]
- **Assessment Objective 1:** Precise recall of detail
- **Assessment Objective 2:** Analytical historical causation

KEY CHRONOLOGY & CAUSAL TURNING POINTS

PEARSON EDEXCEL SPECIFICATION TIMELINE

CHRONOLOGY: PHASE 1 • EARLY DEVELOPMENTS

1123
Rahere founds St Bartholomew's Hospital in London ('Care Not Cure').

↓

C.1242
Ibn al-Nafis discovers the pulmonary transit of blood in Damascus.

↓

1277
Roger Bacon imprisoned by Church authorities for advocating empirical observation.

CHRONOLOGY: PHASE 2 • TURNING POINTS & CRISES

1345
Planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for plague.

↓

1348
The Black Death strikes Melcombe Regis, killing 30–45% of England's population.

↓

1376
Guild of Surgeons established in London, separating surgeons from barbers.

CHRONOLOGY: PHASE 3 • INSTITUTIONAL EVOLUTION

C.1380
MS Harley 3719 illustrates the 'Zodiac Man' (Homo Signorum) for bloodletting.

↓

1400
Over 500 monastic hospitals operate across England under religious orders.

ERA 1 • LESSON 1 PEARSON EDEXCEL GCSE HISTORY • PAPER 1 (1HI0/11)

KT1.1: Supernatural & Religious Explanations of Disease (c1250–c1500)

Specification Anchor: Ideas about the cause of disease and illness: supernatural and religious explanations; the influence of the Church on medicine and science.

The Catholic Church Monopoly

[1.1] In medieval England (c1250–c1500), the Roman Catholic Church exercised an unshakeable institutional monopoly over intellectual life, education, and societal values. Formal scholarship was an ecclesiastical privilege; cathedral grammar schools and universities like Oxford, Cambridge, and Paris were staffed exclusively by tonsured Catholic clergy. Most decisively, all book production and manuscript dissemination were concentrated within monastic scriptoria. In these monastic copying rooms, Benedictine and Cistercian monks laboriously hand-transcribed Latin scrolls onto vellum parchment over months of physical toil. Because the Church controlled the means of textual reproduction, clerical superiors operated as unyielding censors, deciding precisely which ancient treatises were copied, translated, or permanently suppressed from university libraries.

[1.2] Medical instruction across medieval European faculties was strictly theoretical, scholastic, and subordinated to biblical scripture. The Church taught that the material universe was a temporary, imperfect terrestrial realm entirely subservient to divine providence. Consequently, comprehending human anatomy did not require physical dissection, clinical experimentation, or empirical inquiry; it demanded the memorisation of holy scripture alongside ancient philosophical authorities sanctified by papal decree. Within medieval theological orthodoxy, sickness was treated as a spiritual condition reflecting divine purpose. To openly dispute canonical authority or propose alternate physical causes of disease was not treated as academic disagreement—it was prosecuted as dangerous heresy against Almighty God.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

The Doom Painting of St Thomas's Church, Salisbury (c.1475)



> *"The monumental 15th-century Doom painting above the chancel arch depicting Christ in Majesty, the righteous ascending to Heaven (left), and the damned being dragged into the Hellmouth (right)."*

Provenance: St Thomas's Church, Salisbury, c.1475 wall mural above the chancel arch.

Hinge Question: *Study Christ judging humanity and the damned being dragged into the Hellmouth on the right. How did medieval wall paintings like this reinforce the belief that sickness and epidemics were*

God's Wrath, Sin & Leprosy

[2.1] Because Catholic dogma taught that God held omnipotent sovereignty over human existence, sickness was universally interpreted through a supernatural framework. An outbreak of sudden disease or epidemic pestilence was regarded as divine retribution—a deliberate punishment unleashed by God to chastise individuals or whole communities for mortal sins such as pride, gluttony, blasphemy, or heresy. Sickness could also be interpreted as an intentional divine test of spiritual fortitude, directly mirroring the suffering of Job in the Old Testament. Because the ultimate origin of disease was divine wrath, recovery was viewed as a spiritual pardon granted through prayer, penitence, fasting, and intercessory Masses, rather than physical medical intervention.

[2.2] The most horrifying manifestation of divine punishment in the medieval imagination was leprosy. Prominently chronicled in scripture, leprosy was regarded as the outward physical rotting of an inwardly corrupted, sinful soul. When an individual was diagnosed, church authorities performed a solemn ritual of separation (sequestratio), during which a priest cast consecrated churchyard earth over the sufferer's shoes, formally pronouncing them dead to the secular world. Lepers were cast out of towns and banished to isolated Lazar houses (leprosaria) constructed outside municipal walls. When walking country tracks, lepers were compelled by law to wear coarse grey hooded cloaks, walk on the leeward side of the road, and continuously rattle a wooden clapper or bell to warn healthy travelers to avoid their foul breath.

PRIMARY EXCERPT PHILOSOPHICAL TREATISE & HERESY CHARGE 1267

Empirical Observation vs Ecclesiastical Dogma

> *"The strongest argument proves nothing so long as the conclusions are not verified by experience of an external kind. If we wish to know the truth of things, we must examine natural objects with our own senses, not bow down to ancient pagan authors or the decrees of schoolmen."*

Roger Bacon • Opus Majus (Oxford & Paris, 1267) 1267

Hinge Question: *Why did Bacon's insistence on experiential testing lead*

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS CORE VOCABULARY

the medieval Catholic Church hold a monopoly on medical knowledge: The Church controlled all universities and monastic scriptoria, where monks hand-copied books and strictly censored ideas that contradicted scripture.

medieval people explain the religious cause of disease: Illness was viewed as divine retribution sent by God to punish sin, test faith, or cleanse the soul; recovery was achieved through prayer and repentance.

Lazar houses: Segregated quarantine colonies built outside town boundaries to isolate leprosy sufferers, who were forced to wear cloaks and ring warning bells.

Celestial Forces & Astrology

[3.1] Alongside divine wrath, medieval society believed that human physiological health was intimately dictated by celestial configurations. While early Christian theologians had initially rejected astrological horoscopes as pagan divination, by the fourteenth century astrological medicine had become fully institutionalized within university medical faculties. European scholars conceptualized the cosmos as God's grand clockwork mechanism, wherein the movement of the seven celestial spheres exerted continuous metaphysical and physical forces upon terrestrial matter. Medical practitioners carried folding pocket almanacs called Vademecums, which contained diagrams of the Zodiac Man (Homo Signorum), linking the twelve astrological constellations to specific zones of the human anatomy.

[3.2] Whenever inexplicable continental epidemics struck, medieval physicians consulted astrological ephemerides to locate the causal celestial rupture. Most famously, when the Black Death swept across Western Europe in 1348, the royal medical faculty at the University of Paris issued an authoritative report declaring that the catastrophe originated on 20 March 1345 from a rare planetary conjunction of Saturn, Jupiter, and Mars in the fourteenth degree of Aquarius. Physicians argued that the malevolent heat of Mars combined with the corrupting moisture of Jupiter had drawn up toxic subterranean vapors from the earth, poisoning the atmospheric air with pestilential miasma. Consequently, physicians checked lunar charts before performing phlebotomy, fearing that bleeding a patient under the wrong zodiac sign would prove instantly fatal.

SOURCE B ARCHIVAL RECORD & ANALYSIS c1250–c1500

The 'Zodiac Man' (Homo Signorum, c.1380)



"The Zodiac Man (Homo Signorum) showing the 12 astrological signs that medieval physicians believed governed different bodily organs."

Provenance: MS Harley 3719, British Library, London, c.1380.

Hinge Question: *Notice how each astrological constellation rules a specific part of the human body (such as Aries governing the head and Pisces governing the feet). Why did medieval physicians carry pocket almanacs and consult the position of the moon before performing bloodletting or surgery on a patient?*

Galen's Teleology & The Enforcement of Dogma

[4.1] Why did the medieval Christian Church grant supreme, unquestioned authority to Claudius Galen, a pagan physician who practiced in pagan imperial Rome during the second century AD? The fundamental explanation was teleology. In his voluminous anatomical and philosophical treatises, Galen repeatedly argued that every single organ, bone, muscle, and tendon in the human body had been purposefully crafted with anatomical perfection by a supreme, benevolent, and singular Creator. This teleological design argument aligned so flawlessly with Christian creation theology and the Book of Genesis that the Papacy and university faculties embraced Galen's writings as sacred, divinely inspired truth.

[4.2] The resulting consequence for European medical development was centuries of profound intellectual stagnation. Questioning Galen's anatomical descriptions was treated as questioning the perfection of God's divine design. During rare university dissections of executed felons, the scholarly physician (lector) sat aloft in an elevated pulpit reading aloud from Galen's Latin texts, while an unlettered barber-surgeon made crude incisions; if a cadaver's internal organs contradicted Galen, lecturers insisted the corpse was deformed rather than admit Galen was mistaken. When the brilliant English Franciscan friar Roger Bacon argued at Oxford around 1277 that scholars must abandon blind deference to ancient texts and test nature through firsthand empirical experimentation, church authorities condemned his writings and imprisoned him for fourteen years. For over two centuries, scientific inquiry remained effectively outlawed.

KEY FIGURE c.1214–1292



Roger Bacon

FRANCISCAN SCHOLAR & NATURAL PHILOSOPHER

Early advocate of empirical scientific observation who was imprisoned by the Catholic Church for questioning ancient authority.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Advocated Observation:** Argued doctors should test ideas through experimentation rather than dogmatically copying ancient texts.
- **Imprisoned in 1277:** Punished by the Franciscan Order for holding "suspect novelties", deterring empirical science.
- **Symbol of Censorship:** Demonstrated the Church's unyielding enforcement of Galenic and theological orthodoxy.

HISTORICAL CONCEPT THEOLOGICAL DOGMA

Teleology & The Papal Monopoly on Knowledge

Why did the Christian Church adopt the medical treatises of Claudius Galen, a pagan doctor from 2nd-century imperial Rome? The crucial connection was teleology. Galen repeatedly argued in his treatises that every single bone, muscle, and organ in the human body had been purposefully crafted by a supreme, singular Creator. This design argument aligned so perfectly with Genesis that church leaders declared Galen's anatomical writings infallible sacred truth.

Key Causation: Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.

KEY ENQUIRY CHECK • SUPERNATURAL & RELIGIOUS EXPLANATIONS OF DISEASE (C1250–C1500) CHECK YOUR UNDERSTANDING

1. Causal Recall: Explain one key cause explored in this enquiry.

2. Historical Analysis: How did contemporary individuals respond to this challenge?

3. Evaluative Hinge: Assess the extent of change compared to earlier periods.

ERA 1 • LESSON 2 PEARSON EDEXCEL GCSE HISTORY • PAPER 1 (1HI0/11)

KT1.2: Rational Explanations: Hippocrates, Galen & The Four Humours (c1250–c1500)

Specification Anchor: Rational explanations: the Theory of the Four Humours and the Theory of Opposites; the miasma theory; the continuing influence of Hippocrates and Galen.

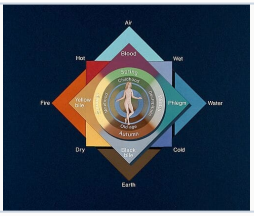
The Greek Foundations of Humoural Theory

[1.1] While the Christian Church explained the ultimate theological purpose behind illness, medieval physicians required a natural, biological framework to diagnose and treat diseases in day-to-day practice. For this physiological model, European universities relied upon the classical corpus of ancient Greek medicine, foremost among which were the writings attributed to Hippocrates of Kos (c. 460–c. 370 BC). Hippocrates achieved a historic breakthrough in ancient thought by explicitly detaching medicine from superstition, demonic possession, and divine wrath, arguing that the human body operated as a physical system governed entirely by natural laws, diet, environmental climate, and bodily fluids.

[1.2] Hippocrates established the foundational doctrine of pathology known as the Theory of the Four Humours. He posited that the human body contained four primary internal liquids, each corresponding to an element, a season, and two fundamental qualities: Blood (originating in the liver, hot and wet, linked to air and spring); Phlegm (originating in the brain and lungs, cold and wet, linked to water and winter); Yellow Bile or Choler (originating in the gallbladder, hot and dry, linked to fire and summer); and Black Bile or Melancholy (originating in the spleen, cold and dry, linked to earth and autumn). Good physical health required these four humours to remain in harmonious internal balance (eukrasia). When any humour became excessive, deficient, or putrefied, acute sickness inevitably followed.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

The Four Humours Wheel (Tacuinum Sanitatis, 14th c.)



"The Four Humours Wheel: Blood (air/hot & wet), Phlegm (water/cold & wet), Yellow Bile (fire/hot & dry), and Black Bile (earth/cold & dry)."

Provenance: Tacuinum Sanitatis manuscript, 14th century, Österreichische Nationalbibliothek, Vienna.

Hinge Question: *Study how each of the four humours (Blood, Phlegm, Yellow Bile, Black Bile) is linked to a season, an element, and specific qualities of heat and moisture. If a patient suffering from a cold and shivering was treated with hot spices and dry foods under Galen's Theory of Opposites, why did this belief in internal fluid balance prevent medieval physicians from searching for external causes of disease?*

Galen & The Theory of Opposites

[2.1] In the second century AD, the Greek physician Claudius Galen, practicing in imperial Rome, adapted Hippocrates' humoural theory into a comprehensive, rigid clinical methodology known as the Theory of Opposites (Contraria Contrariis Curantur). Galen argued that physical disease was not a mysterious invasion from outside, but an internal humoural imbalance that a rational practitioner could systematically counteract. Rather than simply waiting for nature to heal the patient, Galen instructed physicians to actively neutralize the excessive qualities of the dominating humour by administering therapeutic treatments possessing the exact contrary physical properties.

[2.2] Under Galen's Opposites system, diagnosis and prescription followed an unyielding mathematical logic. If a patient presented with a violent shivering winter chill accompanied by excessive white mucous, the physician diagnosed a surplus of cold, wet phlegm; the prescribed remedy consisted of hot, dry foods, spiced wine, ginger, black pepper, and dry bedding near an open hearth. Conversely, if a patient suffered from a burning, inflammatory fever and flushed skin, the diagnosis was an excess of hot, dry yellow bile or hot blood; the treatment demanded cooling cucumber ointments, cold barley broths, and immediate phlebotomy to evacuate internal heat. This mechanical system gave medieval university physicians an immense sense of professional certainty, even though its biological premise was completely false.

PRIMARY EXCERPT CLASSICAL MEDICAL TREATISE c.170 AD

The Law of Opposites & Humoural Pathophysiology

"All diseases are caused by an excess or defect of the four elemental qualities—heat, cold, dryness, and moisture. If a disease is hot and dry, such as a burning pleurisy, you must administer remedies that are cold and moist to restore the body to its natural equilibrium."

Claudius Galen • De Temperamentis (Rome, c.170 AD) c.170 AD

Hinge Question: *How did Galen's Theory of Opposites give medieval physicians a rational clinical framework, even though their physiological premise was completely mistaken?*

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS CORE VOCABULARY

the Four Humours: Blood (hot/wet), Phlegm (cold/wet), Yellow Bile (hot/dry), and Black Bile (cold/dry). Sickness was an internal imbalance of these fluids.

Galen's Theory of Opposites: A clinical method treating excess humours with contrary qualities (e.g. treating cold, wet phlegm with hot, dry peppers and spices).

Miasma Theory: The belief that poisonous, foul-smelling air from swamps, rotting waste, and corpses poisoned the body's humours upon inhalation.

Miasma Theory & Pestilential Air

[3.1] Alongside internal humoural dyscrasia, the dominant environmental explanation for epidemic outbreaks was the Theory of Miasma. Originating in the works of Hippocrates and Galen, miasma was defined as poisonous, corrupt air filled with putrid atmospheric vapors. Medieval physicians believed that inhaling foul-smelling vapors corrupted the internal humours inside the lungs and heart, causing immediate decomposition of the blood. In municipal charters and hospital statutes, this phenomenon was recorded as "pestilential corruption of the atmosphere" or "foul exhalations," establishing a direct causal connection between sensory stench and physical mortality.

[3.2] Contemporary natural philosophy taught that miasma was generated by terrestrial decay: stagnant marshes, open town gutters, unburied decomposing corpses, cesspits overflowing with human excrement, and the noxious entrails of slaughtered livestock dumped into public rivers. Because noxious odors were believed to be actively infectious, medieval disease prevention focused obsessively upon aromatic olfactory barriers. Aristocrats and wealthy burgesses carried silver pomanders stuffed with ambergris, cloves, and musk, held sweet-scented floral posies beneath their noses in crowded streets, and burned fragrant resins like frankincense, rosemary, and juniper within shuttered domestic chambers to purify the atmospheric air.

SOURCE B ARCHIVAL RECORD & ANALYSIS c1250–c1500

Physician Inspecting Urine Flask (Fasciculus Medicinae, 1491)



"The Medieval Urine Chart: a radial wheel showing twenty flasks of differing colours, which physicians examined, smelled, and sometimes tasted to diagnose humoural imbalance."

Provenance: Johannes de Ketham, Fasciculus Medicinae, Venice, 1491.

Hinge Question: *Observe the physician holding up a glass matula (urine flask) to inspect the color, sediment, and clarity of the liquid against a radial wheel. How did uroscopy allow medieval doctors to diagnose internal humoural imbalance without performing surgery or human dissection?*

Diagnostic Tools—Uroscopy & The Vademecum

[4.1] Because human anatomical dissection was forbidden by religious convention and cultural taboo, how did a university-trained medieval physician actually deduce which humour was corrupted within a living patient? The supreme clinical diagnostic instrument was Uroscopy (the examination of urine). Practitioners collected morning urine specimens in a specialized, thin pear-shaped glass flask called a matula, whose shape was designed to mirror the interior cavities of the human bladder. The physician held the matula aloft against the light, scrupulously inspecting its color, opacity, bubbles, and particulate sediment, occasionally smelling or tasting the liquid to assess its internal humoural qualities.

[4.2] The physician evaluated the specimen against a standardized circular urine wheel containing twenty distinct color gradients, ranging from pale water-white through saffron-gold to dark ruby-red and black. A reddish or golden tint signaled an excess of fiery blood or choler; pale, cloudy urine betrayed cold phlegm; and dark, murky sediment demonstrated corrupt black bile. Physicians carried these diagnostic charts folded inside their belt-worn Vademecum handbooks alongside lunar ephemerides. While uroscopy represented a form of empirical clinical observation, it firmly trapped medieval medicine within Galen's flawed humoural dogma. Because physicians attributed every pathological symptom to humoural fluids rather than specific microbial pathogens, medical treatment remained completely incapable of curing genuine physical disease.

KEY FIGURE c.129–c.216 AD



Claudius Galen

ANCIENT ROMAN IMPERIAL PHYSICIAN

Roman doctor whose Theory of Opposites and teleological design argument formed the unquestioned foundation of medieval European medicine.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Theory of Opposites:** Developed treatments balancing the Four Humours using opposing qualities (cold phlegm treated with hot pepper).
- **Teleological Doctrine:** Argued every organ was purposefully designed by a single Creator, perfectly matching Genesis creation theology.
- **Anatomical Dominance:** Dissected apes and pigs, creating errors (e.g. 2-piece jawbone) that remained dogma for over 1,300 years.

HISTORICAL CONCEPT RATIONAL PATHOLOGY

The Humoural Axis: Balance, Phlebotomy & Opposites

Medieval physicians viewed the human body as an organic microcosm governed by the Four Humours: Blood (spring/air/hot & wet), Yellow Bile (summer/fire/hot & dry), Black Bile (autumn/earth/cold & dry), and Phlegm (winter/water/cold & wet). Disease occurred when one humour became excessive or corrupt. Treatment required active subtraction via phlebotomy (veinesection, cupping, or leeches) or clinical purging using emetics and laxatives.

Exam Distinction: Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.

KEY ENQUIRY CHECK • RATIONAL EXPLANATIONS: HIPPOCRATES, GALEN & THE FOUR HUMOURS (C1250–C1500) CHECK YOUR UNDERSTANDING

1. Causal Recall: Explain one key cause explored in this enquiry.

2. Historical Analysis: How did contemporary individuals respond to this challenge?

3. Evaluative Hinge: Assess the extent of change compared to earlier periods.

ERA 1 • LESSON 3 PEARSON EDEXCEL GCSE HISTORY • PAPER 1 (1HI0/11)

KT1.3: Approaches to Prevention & Treatment: Rituals, Bleeding & Purging (c1250–c1500)

Specification Anchor: Approaches to prevention and treatment: religious and supernatural methods (prayer, fasting, pilgrimages); rational treatments (bloodletting, purging, herbal remedies, regimen sanitatis).

Spiritual Healing & Religious Penitence

[1.1] Because medieval society fundamentally believed that health and illness were governed by divine will, the initial and most urgent response to any sickness was spiritual reconciliation. Church authorities instructed that physical remedies were completely futile unless the patient's soul was first cleansed of sin through sacramental confession with a parish priest. Parishioners were warned that seeking medical cures without repenting mortal sins was an act of pride that would invite eternal damnation. Affluent families commissioned monasteries to sing daily intercessory requiem Masses, while common folk fasted rigorously, wore coarse hair-shirts, distributed alms to the destitute, and lit votive wax candles before altar statues of patron saints.

[1.2] Pilgrimage was regarded as the ultimate therapeutic journey in medieval England. Hundreds of thousands of afflicted sufferers walked vast distances along ancient pilgrimage routes to sacred shrines housing miraculous holy relics. Sick pilgrims journeyed to the tomb of Saint Thomas Becket at Canterbury Cathedral, the shrine of Our Lady at Walsingham, or Saint Cuthbert at Durham. Upon reaching the shrine, sufferers knelt in prayer, touched reliquaries encasing holy bones, drank blessed well water, and purchased lead pilgrim badges as protective amulets. Monastic chronicle rolls recorded hundreds of miraculous cures—from restored sight to the healing of tumours—reinforcing the popular conviction that divine mercy was infinitely more efficacious than mortal medicine.

Phlebotomy—The Universal Humoural Treatment

[2.1] Whenever physical therapy was deployed, the most universally practiced and revered procedure was Phlebotomy (bloodletting). Derived directly from Galen's anatomical doctrine that blood was the dominant, warmest humour and the primary combustible fuel for fevers, physicians ordered regular venesection to evacuate excess heat and rebalance bodily fluids. Bloodletting was so completely integrated into daily medieval routine that English monasteries scheduled compulsory "bleeding weeks" (minutio) several times annually, during which entire monastic communities were bled in the infirmary for preventive hygiene, receiving nourishing meat broths to recuperate.

[2.2] Phlebotomy was executed via three distinct physical procedures. The standard technique was venesection, wherein a practitioner used a razor-sharp, spring-loaded or handheld blade called a fleam to slice open an arm or neck vein, collecting measured pints of steaming blood in graded pewter bleeding bowls. For localized inflammation or pain, practitioners performed Cupping: heating a glass or bronze cup over a candle flame and sealing it over scarified skin, creating an airtight vacuum that drew blood into the tissue. Finally, Leeching was utilized for sensitive bodily areas like the throat or gums; medicinal leeches (Hirudo medicinalis) were applied to suck corrupt blood until engorged. While intended to restore equilibrium, excessive bloodletting caused massive blood loss, severe hypovolemic shock, and frequently precipitated the death of already weakened patients.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Phlebotomy / Bloodletting Manuscript (MS Sloane 1977, c.1300)



"A medieval physician opening a vein in the arm using a fleam while blood drains into a bowl, supervised by an attendant."

Provenance: British Library MS Sloane 1977, England, c.1300.

Hinge Question: *Study the physician using a fleam (blade) to open a vein and collect blood in a bowl. Why did medieval people believe that releasing blood was an essential way to restore balance to the body and prevent fevers, even though it frequently weakened the patient?*

PRIMARY EXCERPT MONASTIC MEDICAL REGIMEN 13th Century

Monastic Phlebotomy & The Regimen of Health

"Bleeding clears the mind, cleanses the stomach, warms the marrow, purges the bowels, and produces a long life. Let no brother be bled during the dog days of July or August, nor under an ill-aspected moon, lest the blood be drawn backward into the heart."

Regimen Sanitatis Salernitanum • Medieval Monastic Health Code c.1250

Hinge Question: *Why was bloodletting regarded as essential preventive maintenance in medieval monasteries rather than merely a desperate cure?*

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS CORE VOCABULARY

Phlebotomy: The practice of bloodletting using fleams, cupping glasses, or medicinal leeches to lower internal bodily heat and restore humoural balance.

the three methods of bloodletting: 1. Vein-opening with a fleam; 2. Cupping (heated glass vacuum); 3. Leeching (medicinal leeches drawing blood).

Purging: Using emetics (inducing vomiting) and strong laxatives (scammony, hellebore) or clysters (enemas) to clear corrupt humours from the digestive tract.

Purging, Emetics & Herbal Theriac

[3.1] When phlebotomy failed to relieve humoural congestion, physicians resorted to aggressive Purging to evacuate putrefied matter from the gastrointestinal tract. In medieval physiology, the stomach was conceptualized as a simmering cooking vessel; if food spoiled before complete digestion, it generated toxic yellow bile and melancholic black bile. To clear the upper tract, physicians administered violent emetics containing antimony, copper sulfate, or dried cantharides beetles, inducing prolonged vomiting. To clear the bowels, practitioners prescribed powerful laxatives compounded from scammony, senna, black hellebore, and linseeds, or administered clysters (enemas) using hollow goose quills and pig bladders to pump caustic herbal decoctions into the rectum.

[3.2] Alongside purging, pharmacopoeia formed the primary medicinal therapy, mixed by guild apothecaries from native botanicals and costly imported spices. Household herb gardens supplied mint, sage, chamomile, wormwood, and garlic, which were boiled into bitter syrups, electuaries, and healing poultices. The most celebrated universal antidote was Theriac (Venice treacle), a complex ancient compound tracing back to King Mithridates of Pontus. Containing over sixty disparate ingredients—including crushed viper flesh, roasted scorpions, raw opium, gum arabic, crushed pearls, and honey—Theriac required months of fermentation. Regarded as a miraculous cure-all for poisons, fevers, and plague, Theriac was so profoundly trusted that doctors believed it could neutralize any humoural corruption.

SOURCE B ARCHIVAL RECORD & ANALYSIS c1250–c1500

Portrait of Claudius Galen (Engraving)



> *"Claudius Galen (AD 129–c. 216), whose clinical treatises on the Theory of Opposites, phlebotomy, and purging dominated European medicine for 1,400 years."*

Provenance: Historical engraving after ancient Roman busts of Galen.

Hinge Question: *Galen taught that the humours must be kept in constant equilibrium through balancing treatments. Why did his authoritative treatises lead doctors to prescribe violent purging (emetics and laxatives) alongside herbal compounds like Theriac, rather than searching for specific remedies for individual illnesses?*

Prevention & The Regimen Sanitatis

[4.1] In medieval medical philosophy, preventive preservation of health was regarded as vastly superior to curative intervention. Maintaining personal physical equilibrium was considered an ethical and physical duty. Noblemen and wealthy clerics commissioned personalized parchment copies of the Regimen Sanitatis Salernitanum (The Salerno Rule of Health), an authoritative lifestyle treatise originating from the prestigious medical school of Salerno in southern Italy. The text instructed readers to regulate their daily existence across the "Six Non-Naturals": the quality of ambient air, balance of food and drink, physical exercise and rest, sleep and wakefulness, secretions and excretions, and psychological passions of the soul.

[4.2] Adherents of the Regimen were instructed to rise early, wash their hands and face in cold stream water, comb their hair to clear cerebral vapors, avoid damp night mists, sleep with shuttered windows, and consume foods tailored to their individual humoural temperament. Many wealthy households installed private steam baths or visited municipal public bathhouses (stewes). While these hygiene practices promoted personal cleanliness and general wellbeing, they afforded zero protection against lethal bacterial or viral epidemics. Because all medieval preventive and curative approaches remained anchored to the fictional balance of Four Humours, medical practice remained static for centuries, offering virtually no genuine physiological protection against infectious disease.

KEY FIGURE c.460–c.370 BC



Hippocrates of Kos

ANCIENT GREEK PHYSICIAN & PHILOSOPHER

The "Father of Medicine" who separated disease from supernatural curses and pioneered clinical bedside observation.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Theory of the Four Humours:** Identified Blood, Phlegm, Yellow Bile, and Black Bile as the determinants of physical health.
- **Clinical Observation:** Instructed physicians to examine symptoms, pulse, and urine systematically rather than relying on omens.
- **The Hippocratic Oath:** Established an ethical code of conduct mandating that doctors protect patient confidentiality and "do no harm".

HISTORICAL CONCEPT PREVENTIVE REGIMES

Regimen Sanitatis: Preventive Hygiene & The Six Non-Naturals

Medieval prevention centered on the Regimen Sanitatis, a set of dietary and behavioural rules originating in ancient Greek medicine. Doctors advised patients on the "Six Non-Naturals": air quality, food and drink, motion and rest, sleep and waking, excretion and retention, and passions of the soul. Wealthy citizens burned aromatic sweet woods (frankincense, lavender) to counteract miasma and bathed regularly in stewes.

Key Evidence: Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.

KEY ENQUIRY CHECK • APPROACHES TO PREVENTION & TREATMENT: RITUALS, BLEEDING & PURGING (C1250–C1500) CHECK YOUR UNDERSTANDING

1. Causal Recall: Explain one key cause explored in this enquiry.

2. Historical Analysis: How did contemporary individuals respond to this challenge?

3. Evaluative Hinge: Assess the extent of change compared to earlier periods.

ERA 1 • LESSON 4 PEARSON EDEXCEL GCSE HISTORY • PAPER 1 (1HI0/11)

KT1.4: Medical Care Providers & Monastic Hospitals: 'Care Not Cure' (c1250–c1500)

Specification Anchor: The roles of the physician, apothecary and barber surgeon; care in the home; the role of medieval hospitals.

The Elite University Physician

[1.1] In medieval England, access to formal medical treatment was rigidly stratified by wealth, social hierarchy, and geography. At the supreme apex of the medical hierarchy stood the university-trained Physician (Medicus). To acquire a medical licence, a physician spent between seven and ten years in rigorous post-graduate scholastic study at church-dominated universities such as Oxford, Cambridge, Paris, Bologna, or Montpellier. Their curriculum was overwhelmingly theoretical, philological, and literary; students spent endless terms memorising, debating, and transcribing classical Latin translations of Galen, Hippocrates, and Arabic medical luminaries like Avicenna (Ibn Sina) and Rhazes (al-Razi). Direct physical examination of sick human bodies was virtually nonexistent during their degrees.

[1.2] Because university education was extraordinarily costly and prolonged, licensed physicians were an extreme luxury. Historical tax records indicate that fewer than one hundred fully licensed physicians practiced throughout the entirety of England before 1350. Their astronomical fees meant they were retained almost exclusively by the royal court, dukes, high-ranking bishops, and wealthy monastic abbots. When attending an affluent patient, the physician performed an aristocratic intellectual consultation: he refrained from touching wounds or administering treatments himself, instead inspecting the urine flask (matula), feeling the pulse, calculating astrological planet houses, and writing a complex Latin prescription for lower-status manual tradesmen to execute.

Barber-Surgeons, Apothecaries & Domestic Healers

[2.1] For the remaining ninety-nine percent of the population, physical healthcare was provided by manual craft guilds rather than academic scholars. Hands-on operative procedures were performed by Barber-Surgeons. Bound within urban livery companies and trained through strenuous seven-year craft apprenticeships, barber-surgeons operated with razors, fleams, probes, tooth-keys, and bone-saws. They extracted rotten teeth, dressed weeping ulcers, stitched lacerations, set fractured bones, lanced inflamed boils, and performed phlebotomy. Because they performed manual labor and soiled their fingers with human blood, university physicians regarded them with aristocratic condescension, classifying them as uneducated artisans.

[2.2] Medicinal remedies were compounded and dispensed by Apothecaries. Organized in merchant guilds like London's Worshipful Company of Grocers, apothecaries mixed medicinal syrups, electuaries, and salves from wild English plants and costly Mediterranean spices. Yet the true foundational bedrock of medieval medical relief was domestic care within the home, administered by mothers, housewives, and local village "wise women." Women brewed herbal infusions, applied soothing poultices to feverish children, and acted as community midwives. Local wise women preserved centuries of herbal folklore, diagnosing common ailments and providing affordable, compassionate care to agrarian peasant villages completely abandoned by licensed medicine.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Medieval Surgeon Suturing a Head Wound (14th c.)



> *"A guild-trained surgeon physically stitching a patient's severe head wound with needle and thread, recorded in a 14th-century Latin surgical treatise."*

Provenance: Wellcome Collection MS L0037333, 14th century surgical treatise.

Hinge Question: *Observe the surgeon physically stitching a wound without any antiseptics or anaesthetics. Why were barber-surgeons considered socially inferior to university-trained physicians in the Middle Ages, despite being the only practitioners performing practical, hands-on treatment?*

PRIMARY EXCERPT HOSPITAL FOUNDATION CHARTER 1123

Foundation Charter of St Bartholomew's Hospital

> *"We have founded this hospital for the reception of poor sick persons, that they may be refreshed with food and drink, cheered by the ministry of the brethren, and commended to the mercy of Almighty God, till they be restored to health or gathered unto their fathers in peace."*

Rahere • Foundation Charter of St Bartholomew's Hospital, Smithfield (London, 1123) 1123

Hinge Question: *How does this charter prove that medieval English hospitals were spiritual hospices for palliative care rather than places of surgical or medical cure?*

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS CORE VOCABULARY

the training of a medieval university physician: 7 to 10 years of theoretical book-learning at universities like Oxford or Paris, studying Latin translations of Galen and Hippocrates, with almost zero hands-on clinical practice.

the role of a barber-surgeon: Guild-apprenticed craftsmen who performed manual surgery: bloodletting, pulling teeth, suturing wounds, lancing boils, and amputations without anaesthetics.

What did apothecaries do: Guild merchants who mixed and sold herbal remedies, ointments, and syrups based on traditional recipes and Galen's Theory of Opposites.

The Medieval Monastic Hospital

[3.1] Between 1100 and 1500, over 1,200 hospitals were founded across the towns and shires of England. However, a medieval hospital was fundamentally distinct from a modern clinical healing institution. The English word "hospital" derived directly from the Latin concept of hospitalitas—the Christian virtue of hospitality toward the stranger and the destitute. Medieval hospitals were ecclesiastical charities established by royal charters, wealthy merchant guilds, or aristocratic patrons seeking spiritual salvation and reduction of their years in Purgatory. They were governed and staffed exclusively by Catholic religious orders—monks, Augustinian canons, and nuns—not by doctors or surgeons.

[3.2] Famous urban foundations like St Bartholomew's (chartered in London in 1123 by the courtier Rahere) and St Thomas's provided sanctuary for weary pilgrims, the infirm elderly, and the impoverished sick. The overriding operational mission was "Care Not Cure." Inmates were washed, clothed in clean linen, warmed beside massive roaring stone hearths, fed wholesome meat broths and fresh ale, and comforted through continuous monastic chanting. However, no medical surgeries or scientific pharmacological cures were ever attempted. Hospital wards were deliberately designed like church naves, with beds aligned in parallel rows facing an elevated high altar so that bedridden patients could witness the elevation of the consecrated Host during the daily Catholic Mass and pray for the salvation of their immortal souls.

Strict Exclusion & The Lazar House

[4.1] Because the primary spiritual objective of monastic hospitals was peace, liturgical devotion, and quiet charitable sanctuary, hospital masters enforced strict entry exclusions. Institutional foundation statutes explicitly prohibited the admission of persons suffering from contagious infectious fevers, open bleeding ulcers, leprosy, or mental lunacy, as well as pregnant women awaiting childbirth. Monastic administrators feared that howling, delirious, or visibly pestilential patients would disrupt choral prayers and introduce corrupt atmospheric miasma into the sacred precinct.

[4.2] Instead, infectious victims were forcibly segregated outside town ramparts. Sufferers of leprosy—viewed as carrying the physical mark of divine judgment—were confined to isolated Lazar Houses (leprosaria) constructed along arterial highways, such as St Giles-in-the-Fields near London or Sherburn Hospital outside Durham. Lepers wore hooded cloaks and sounded wooden clappers to warn approaching wayfarers of their corrupting presence. In historical evaluation, medieval institutional care represented an extraordinary monument of Christian charity, community shelter, and spiritual solace for the medieval pauper, but contributed absolutely nothing to anatomical discovery or clinical therapeutics. Genuine medical cure remained completely absent from the medieval hospital.

SOURCE B ARCHIVAL RECORD & ANALYSIS c1250–c1500

Hôtel-Dieu Hospital Ward (Livre de Vie Active, c.1482)



"The main ward of the Hôtel-Dieu in Paris: Augustinian nuns tending to rows of patients sharing beds, providing spiritual comfort and rest."

Provenance: Livre de Vie Active des Religieuses de l'Hôtel-Dieu de Paris, Jean Henry, c.1482.

Hinge Question: *Look at the nuns tending to patients in the ward, with an altar visible in the background. Why was the primary purpose of a medieval hospital hospitality and salvation rather than medical cure, and why were infectious patients strictly excluded?*

KEY FIGURE 1307–1392



John of Arderne

MASTER SURGEON OF THE HUNDRED YEARS WAR

Renowned English military surgeon who developed innovative surgical techniques for treating anal fistulae in knights.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Fistula-in-ano Procedure:** Devised a revolutionary surgical incision that achieved a 50% survival rate without fatal hemorrhage.
- **Empirical Practice:** Founded the Guild of Surgeons (1376) and advocated for practical battlefield experience over university theory.
- **Bedside Demeanour:** Wrote *Practica*, advising surgeons to dress soberly, soothe patient anxieties, and agree fees in advance.

HISTORICAL CONCEPT INSTITUTIONAL CARE

'Care Not Cure': The Monastic Hospital Philosophy

Medieval English hospitals were not medical centers designed for surgery or infectious healing. Run by monastic religious orders, their primary purpose was spiritual hospitality and palliative care. Priests and nuns provided clean bedding, warmth, simple food, and daily prayer so patients could die in a state of Christian grace. Infectious lepers, lunatics, pregnant women, and terminal sufferers were routinely turned away.

Core Specification Anchor: "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.

KEY ENQUIRY CHECK • MEDICAL CARE PROVIDERS & MONASTIC HOSPITALS: 'CARE NOT CURE' (C1250–C1500) CHECK YOUR UNDERSTANDING

1. Causal Recall: Explain one key cause explored in this enquiry.

2. Historical Analysis: How did contemporary individuals respond to this challenge?

3. Evaluative Hinge: Assess the extent of change compared to earlier periods.

ERA 1 • LESSON 5 PEARSON EDEXCEL GCSE HISTORY • PAPER 1 (1HI0/11)

KT1.5: Case Study: Dealing with the Black Death (1348–1349)

Specification Anchor: Case study: dealing with the Black Death, 1348–49; approaches to treatment and attempts to prevent its spread.

The Arrival & Deadly Symptoms of the Pestilence

[1.1] In late June 1348, a merchant vessel from Gascony carrying barrels of wine dropped anchor at the southern English port of Melcombe Regis in Dorset. Unbeknownst to the dockers who unloaded the cargo, the bilge timbers and sacking harboured black ship rats (Rattus rattus) infested with tropical rodent fleas (Xenopsylla cheopis), themselves gorged on the virulent bacillus Yersinia pestis. Within weeks, the catastrophic pandemic known to chroniclers as the "Great Pestilence" or "Great Mortality"—and later named the Black Death—ignited across the Dorset countryside. Propelled along overland trade corridors and coastal shipping lanes, it engulfed Bristol, Salisbury, and London by Michaelmas 1348. Between 1348 and 1350, the pestilence obliterated between thirty and fifty percent of the English population, slaughtering an estimated 1.5 to 2 million men, women, and children.

[1.2] The disease struck terror through two distinct, devastating clinical manifestations. The predominant strain was Bubonic Plague, transmitted directly through the bite of an infected rat flea regurgitating bacteria into the human lymphatic system. Sufferers experienced abrupt chills, delirium, agonizing headaches, and subcutaneous haemorrhages that formed dark purple and black petechiae—known morbidly as "God's tokens." Within hours, violent inflammatory swellings called buboes erupted in the lymph nodes of the groin, armpits, and neck, rapidly swelling to the size of apples, blackened with necrosis and oozing putrid matter. Untreated bubonic victims died in agony within three to five days. The second, even more terrifying manifestation was Pneumonic Plague, occurring when the bacterial infection invaded the lungs. Spread directly through aerosolized droplets in breath, sneezes, and bloodstained sputum, pneumonic victims coughed frothy black blood and succumbed within forty-eight hours with a mortality rate approaching one hundred percent.

SOURCE A CONTEMPORARY VISUAL EVIDENCE c1250–c1500

Citizens of Tournai Burying Plague Victims (1349)



"The citizens of Tournai carrying wooden coffins to mass communal plague burial pits during the devastating Black Death epidemic of 1349."

Provenance: Illumination by Pierart dou Tielt, Gilles Li Muisis chronicle, MS

Explaining the Apocalypse—God, Astrology & Miasma

[2.1] Devoid of any concept of microbiology or microscopic pathogen vectors, medieval society rationalized the existential horror through established theological and humoural frameworks. The supreme orthodox explanation was Supernatural: the Church proclaimed that God had unleashed the plague as divine retribution to chastise humanity for mortal sins—notably luxury, avarice, fornication, and blasphemy. Preachers declared that the Day of Judgment was at hand. Concurrently, academic scholars pointed upward to the heavens: the prestigious Medical Faculty of the University of Paris issued an authoritative pronouncement in October 1348 blaming an extraordinary triple planetary conjunction of Saturn, Jupiter, and Mars in the sign of Aquarius in March 1345, which they claimed had drawn up corrupting vapours from the ocean depths.

[2.2] At the terrestrial level, the universal culprit was Miasma Theory. Physicians asserted that noxious, pestilential atmospheric fog—poisoned by rotting unburied cadavers, stagnant open sewers, putrefying animal intestines, and foul volcanic gases—was drawn directly into the lungs, instantly unbalancing the Four Humours and causing internal blood to boil. Across continental Europe, profound mass hysteria ignited anti-Semitic pogroms, with Jewish communities falsely accused of poisoning civic wells and burnt alive at Strasbourg and Mainz. In England, where the Jewish population had already been banished by King Edward I in 1290, popular panic turned toward wandering beggars, lepers, and foreigners.

PRIMARY EXCERPT OFFICIAL MEDICAL FACULTY REPORT October 1348

The Great Planetary Conjunction & Miasma

"We say that the distant and first cause of this pestilence was the conjunction of the three upper planets—Saturn, Jupiter, and Mars—in the fourteenth degree of Aquarius on 20 March 1345. This conjunction attracted from the earth poisonous vapors

KEY SPECIFICATION TERMINOLOGY & DISCIPLINARY CONCEPTS CORE VOCABULARY

the Black Death enter England: June 1348 at the port of Melcombe Regis in Dorset, arriving on trade ships from Gascony carrying plague-infected black rats and fleas.

the two strains of the Black Death: 1. Bubonic plague (transmitted by rat flea bites, causing agonizing groin/armpit buboes); 2. Pneumonic plague (airborne via coughs/breath, attacking lungs, nearly 100% fatal).

believed causes of the Black Death in 1348: God's punishment for sin, the 1345 planetary conjunction of Mars, Jupiter, and Saturn in Aquarius, and corrupt miasma from rotting filth and unburied corpses.

Desperate Treatments & Religious Panic

[3.1] Because society fundamentally viewed the pandemic as divine wrath, the primary public countermeasures were frantic acts of religious penitence. The Crown and church bishops commanded continuous parish Masses, mandatory communal fasting, and massive penitential processions through city streets. Most dramatically, bands of Brotherhood Flagellants marched between towns across continental Europe; stripped to the waist and chanting hymns of remorse, they whipped their bare shoulders with iron-tipped scourges three times daily until blood splattered the cobbles. Tragically, these massive public processions and gatherings directly accelerated the spread of aerosolized pneumonic plague and flea infestation from parish to parish.

[3.2] Medical practitioners proved totally impotent, offering treatments that were futile and agonizing. Barber-surgeons applied heated cupping glasses to buboes or sliced them open with cautery irons to drain "corrupt matter," causing agonizing blood poisoning and accelerating fatal septic shock. Desperate folk remedies abounded: healers strapped live, plucked chickens or dried toads to swollen buboes to "draw the venom into the beast." Sufferers were forced to drink melted gold, powdered emeralds, and potent Theriac concoctions, while houses were fumigated with sulfur, pitch, and dried rosemary to dispel poisonous atmospheric miasma.

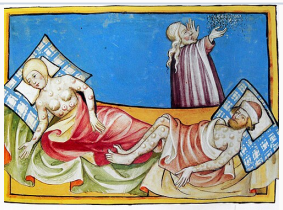
Civic Action, Edward III & Social Collapse

[4.1] In a desperate attempt to fend off miasma, civic leaders and citizens adopted aggressive environmental measures. People walked through alleys holding sweet posies, dried lavender, or sponges steeped in vinegar to their nostrils, while pitch fires burned perpetually at crossroads to purify the air. Certain urban settlements attempted quarantine: the borough of Gloucester sealed its town gates against all outsiders, though country lanes soon brought the infection regardless. In London, parish graveyards filled so rapidly that Bishop Ralph Stratford purchased land at No Man's Land and East Smithfield to excavate colossal communal trench pits, where decaying corpses were stacked like cordwood in layers five deep.

[4.2] In April 1349, King Edward III dispatched an urgent royal mandate to the Mayor of London, furiously denouncing the filthy conditions of the capital where human dung, horse carcasses, and butchers' entrails fouled the streets, generating a deadly stench that threatened his royal court. Edward demanded immediate civic street-clearing to dispel the pestilential miasma. However, because authorities lacked an organized sanitation bureaucracy or police enforcement, and remained wholly ignorant of the rodent and flea vector, royal decrees proved entirely powerless. The Black Death exterminated up to half the population, dismantled feudal manorial obligations, sparked severe agricultural labour shortages, and stood as indelible historical testimony to the utter failure of medieval medicine.

SOURCE B ARCHIVAL RECORD & ANALYSIS c1250–c1500

Plague Victim with Groin and Armpit Buboes (1497)



> *"A bedridden plague victim showing painful inflamed lymph swellings (buboes) in the groin and neck, surrounded by weeping attendants."*

Provenance: Woodcut of St. Sebastian interceding for plague victims, Augsburg, 1497.

Hinge Question: *Observe the painful, dark swellings (buboes) in the groin, armpits, and neck of the plague victim. Why did medieval physicians believe these buboes were proof that poisonous humours were corrupting the body, and why did desperate treatments like lancing them or applying toads fail to cure the disease?*

KEY FIGURE c.1300–1368



Guy de Chauliac

PAPAL PHYSICIAN TO POPE CLEMENT VI

Leading French surgeon who treated victims of the Black Death at Avignon and authored the surgical textbook Chirurgia Magna.

STRATEGIC DECISIONS & CONTRIBUTIONS:

- **Identified Plague Types:** Distinguished clinically between pneumonic plague (blood-spitting) and bubonic plague (buboes).
- **Protected the Papacy:** Advised Pope Clement VI to isolate himself between two roaring fires in Avignon, successfully saving his life.
- **Admitted Medical Powerlessness:** Openly recorded in 1348 that doctors were helpless, as "all medicines and bloodletting availed nothing".

HISTORICAL CONCEPT EPIDEMIOLOGY

Astrology, Miasma & Contagion: The 1348 Conjunction

When the Black Death arrived in June 1348, medieval society deployed three concurrent causal models. Astrologers blamed the March 1345 planetary conjunction of Saturn, Jupiter, and Mars in Aquarius. Churchmen declared it God's vengeance for England's pride and sinful dress. Common folk suspected poisoned wells and foul miasma. Local authorities reacted with quarantine (40-day isolation in Gloucester) and cemetery bans outside city limits.

Historiographical Reality: Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

KEY ENQUIRY CHECK • CASE STUDY: DEALING WITH THE BLACK DEATH (1348–1349) CHECK YOUR UNDERSTANDING

1. Causal Recall: Explain one key cause explored in this enquiry.

2. Historical Analysis: How did contemporary individuals respond to this challenge?

3. Evaluative Hinge: Assess the extent of change compared to earlier periods.

MEDIEVAL MEDICINE IN BRITAIN: REVISION SPINE & EXAM STRATEGY (c1250–c1500)

Comprehensive revision index of pivotal chronology, core specification concepts, and Edexcel examination question frameworks.

MEDIEVAL MEDICINE IN BRITAIN • PIVOTAL CHRONOLOGICAL SEQUENCE & CAUSAL TURNING POINTS

PAPER 1 CHRONOLOGY RECALL

Date	Pivotal Milestone	Historical Significance & Causal Consequence
1123	**Rahere founds St Bartholomew's Hospital in London ('Care Not Cure').**	Rahere founds St Bartholomew's Hospital in London ('Care Not Cure').
c.1242	**Ibn al-Nafis discovers the pulmonary transit of blood in Damascus.**	Ibn al-Nafis discovers the pulmonary transit of blood in Damascus.
1277	**Roger Bacon imprisoned by Church authorities for advocating empirical observation.**	Roger Bacon imprisoned by Church authorities for advocating empirical observation.
1345	**Planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for plague.**	Planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for plague.
1348	**The Black Death strikes Melcombe Regis, killing 30–45% of England's population.**	The Black Death strikes Melcombe Regis, killing 30–45% of England's population.
1376	**Guild of Surgeons established in London, separating surgeons from barbers.**	Guild of Surgeons established in London, separating surgeons from barbers.
c.1380	**MS Harley 3719 illustrates the 'Zodiac Man' (Homo Signorum) for bloodletting.**	MS Harley 3719 illustrates the 'Zodiac Man' (Homo Signorum) for bloodletting.
1400	**Over 500 monastic hospitals operate across England under religious orders.**	Over 500 monastic hospitals operate across England under religious orders.

CORE SPECIFICATION CONCEPTS (C1250–C1500)

- **Teleology & The Papal Monopoly on Knowledge:** Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.
- **The Humoural Axis:** Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.
- **Regimen Sanitatis:** Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.
- **'Care Not Cure':** "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.
- **Astrology, Miasma & Contagion:** Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

EDEXCEL PAPER 1 EXAMINATION FRAMEWORK

SECTION B • 32 MARKS

Q3: Explain one similarity / difference between... [4 Marks]
Strategy: Identify 1 valid conceptual link across eras • Support with precise detail from both specified periods.

Q4: Explain why [change/continuity occurred]... [12 Marks]
Strategy: 3 PEEL paragraphs • Use 2 stimulus points + own knowledge • Link with causal connectives.

Q5/Q6: Evaluative Essay [16 Marks + 4 SPaG]
Strategy: Criteria-led introduction • 3 balanced PEEL paragraphs weighing stated factor vs alternatives • Weighed verdict.

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